

Cerebrolysin

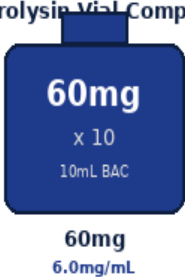
Neuropeptide Complex | 60mg x 10 Vials

Neuroregeneration | Cognitive Protection | IV / IM Injection

Purity: 99.8% | Endotoxin-Free | Research Use Only

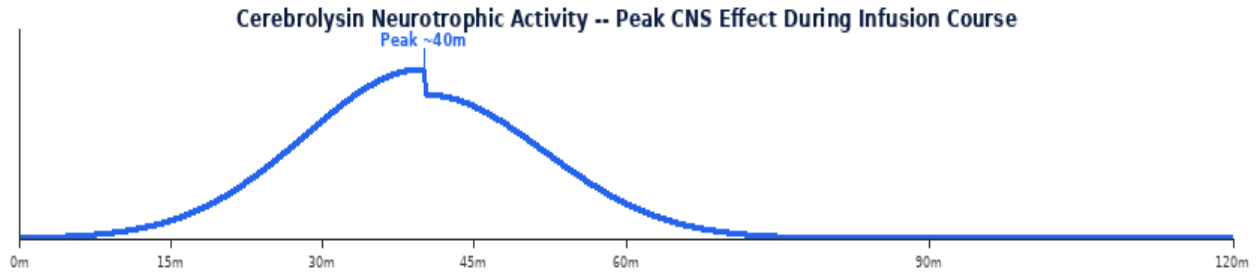
Cerebrolysin is a standardised neuropeptide complex derived from porcine brain tissue. It contains active neurotrophic fragments including BDNF, CNTF, GDNF, and NGF analogues, plus free amino acids (~25% of the preparation). It is the most extensively researched neuroprotective agent with over 40 years of clinical use in Europe, Asia, and Russia for Alzheimer's disease, stroke, TBI, and vascular dementia.

Cerebrolysin Vial Comparison



Parameter	Specification
Class	Standardised porcine brain neuropeptide complex -- neurotrophic factors + amino acids
Vial Size	60mg x 10 vials reconstitute with 10mL saline Purity: standardised
Reconstitution	Add 10mL 0.9% NaCl (saline) -> 6mg/mL do NOT use BAC Water
Standard Dose	1,000-10,000 mcg (1-10mL) per session IV or IM
Routes	IV drip (preferred, 250mL saline over 30-60 min) IM (deltoid/gluteal)
Frequency	Daily x 10-20 days per course IV preferred for neurological protocols
Cycle & Storage	10-20 day courses, 1-2x/year 2-8 deg C use within 24hr reconstituted

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
Neurotrophic Factors	Contains active low-molecular-weight fragments of BDNF, CNTF, GDNF, and NGF analogues that cross the blood-brain barrier; directly stimulates neuronal survival and plasticity; promotes synaptogenesis and dendrite outgrowth	Neuroprotection; neuroregeneration; neuroplasticity research
Alzheimer's & Dementia	Reduces amyloid beta toxicity; decreases tau phosphorylation; promotes cholinergic neuron survival; improves cognitive scores in multiple randomised controlled trials; largest evidence base of any neuropeptide preparation	Alzheimer's disease; vascular dementia; cognitive decline research
Stroke & TBI Recovery	Reduces infarct volume and neurological deficit in ischaemic stroke; accelerates functional recovery post-TBI; anti-excitotoxic (reduces glutamate toxicity); anti-apoptotic via BDNF/BCL-2 pathway; approved for stroke treatment in many countries	Stroke rehabilitation; TBI recovery; ischaemia-reperfusion injury research

Benefit	Context
Largest Evidence Base	Cerebrolysin has over 40 years of clinical use with RCT evidence in Alzheimer's disease, stroke, TBI, and vascular dementia.
Multi-Factor Neuroprotection	Simultaneously delivers BDNF, GDNF, CNTF, and NGF analogues -- no single peptide replicates this multi-factor neurotrophic profile.
BBB Penetration	The low-molecular-weight peptide fragments (<10,000 Da) cross the blood-brain barrier, enabling direct neuronal action after IV/IM administration.

RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



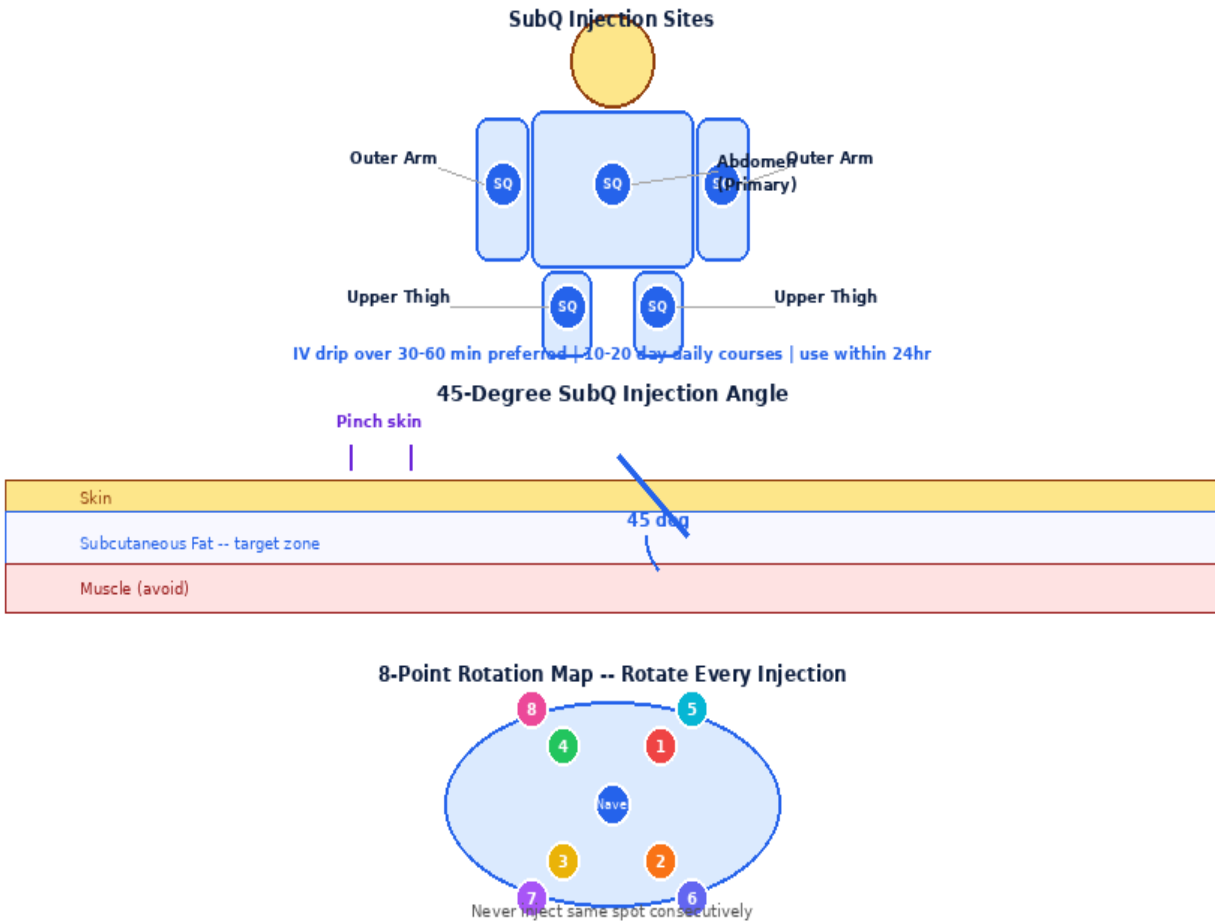
1mL Insulin Syringe | 29-31G | SubQ

Formula: $\text{Vol(mL)} = \frac{\text{Dose(mcg)}}{\text{Conc(mcg/mL)}}$ | **Units:** $\text{Vol} \times 100$

60mg x 10 -- 10mL BAC -> 6.0mg/mL (6,000mcg/mL)

Dose(mcg)	1000mcg	2000mcg	5000mcg	10000mcg
Vol(mL)	0.1667	0.3333	0.8333	1.6667
Units	16.7U	33.3U	83.3U	166.7U
Doses/Vial	6	3	1	0

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



Step	Action	Detail
1	Reconstitute	Add 10mL 0.9% saline (NOT BAC Water) to 60mg vial. Swirl gently. Solution should be clear to slightly yellow.
2	IV drip (preferred)	Dilute in 250mL 0.9% NaCl. Infuse over 30-60 minutes. Infusion rate: slow and steady. Monitor during infusion.
3	IM option	Draw 1-5mL into 23-25G x 1-1.5in syringe. Inject into deltoid or gluteal. Inject slowly. Aspirate first.
4	Use within 24hr	Reconstituted Cerebrolysin degrades rapidly. Prepare fresh each session. Discard all unused solution.

Protocol	Dose	Frequency	Duration	Notes
Cognitive Research	5-10mL per session	Daily x 10-20 days (IV drip)	1-2 courses per year	RCT-validated course protocol for cognitive research.
Neuroprotection / TBI	10mL per session	Daily x 10-20 days (IV)	1-2 courses per year	Full vial per session. IV drip over 30-60 min.
IM Maintenance	1-5mL per injection	Daily x 10-20 days (IM)	Ongoing as needed	Practical alternative to IV for outpatient use.

Side Effect	Likelihood	Management
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Injection site pain (IM)	Common	Warm to room temp. Inject slowly. Rotate deltoid/gluteal.
Headache / dizziness	Occasional	Rate-related (IV). Slow infusion rate.
Agitation / restlessness	Occasional	Dose-related. Reduce dose. Particularly with rapid infusion.
Seizure risk (epilepsy)	Known risk	Contraindicated in epilepsy and status epilepticus.

Contraindications: Cancer history, pregnancy, under 18.

1. 60mg vial: add 10mL 0.9% SALINE (not BAC Water) -> 6mg/mL.
2. CRITICAL: Use within 24 hours of reconstitution -- Cerebrolysin degrades rapidly.
3. PREFERRED ROUTE: IV drip -- dilute in 250mL saline, infuse over 30-60 minutes.
4. Never administer as a rapid IV bolus -- risk of agitation and adverse CNS effects.
5. IM injection is acceptable for lower doses (1-5mL per injection).
6. Administer as 10-20 day daily courses with 1-2 courses per year.
7. Contraindicated in epilepsy, renal failure, and known hypersensitivity.
8. Warm vial to room temperature before injection to reduce IM site pain.
9. Store 2-8 deg C. Protect from freezing. Discard unused reconstituted solution.
10. Not for use during pregnancy, lactation, or in individuals under 18.

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